



**Social Health  
Authority**

# **ACCOUNTING POLICY PAPER INITIAL RECOGNITION & CONTRACT BOUNDARY**

## **SOCIAL HEALTH AUTHORITY**

22<sup>nd</sup> May 2026

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IFRS 17 Insurance Contracts establishes principles for the recognition, measurement, presentation and disclosure of insurance contracts within the scope of the Standard. The objective of IFRS 17 is to ensure that an entity provides relevant information that faithfully represents those contracts. This information gives a basis for users of financial statements to assess the effect that insurance contracts have on the entity's financial position, financial performance and cash flows.

# DOCUMENT TRACKING

## Document History

| Version | Author                                           | Date                       | Revision |
|---------|--------------------------------------------------|----------------------------|----------|
| 1.0     | Kenbright Actuarial & Financial Services Limited | 23 <sup>rd</sup> June 2025 | Creation |
| 2.0     | Kenbright Actuarial & Financial Services Limited | 22 <sup>nd</sup> May 2026  | Final    |

## This Document Has Been Submitted To

| Submitted to            | Date Submitted            |
|-------------------------|---------------------------|
| Social Health Authority | 22 <sup>nd</sup> May 2026 |

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Social Health Authority\IFRS 17 implementation Support\ Social Health Authority-Accounting Policy Paper- Initial Recognition and Contract Boundary\_(220526)

## DOCUMENT PURPOSE

Submission to the Management of Social Health Authority.

## DOCUMENT OWNERSHIP

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# TABLE OF CONTENTS

|          |                                                 |          |
|----------|-------------------------------------------------|----------|
| <b>1</b> | <b>INTRODUCTION .....</b>                       | <b>5</b> |
|          | Background .....                                | 5        |
|          | Purpose.....                                    | 5        |
|          | Objective.....                                  | 5        |
|          | Scope .....                                     | 5        |
| <b>2</b> | <b>INITIAL RECOGNITION.....</b>                 | <b>6</b> |
|          | Application.....                                | 6        |
|          | Recognition Point by Category .....             | 6        |
| <b>3</b> | <b>DETERMINATION OF CONTRACT BOUNDARY.....</b>  | <b>7</b> |
|          | Application to SHA's operating Environment..... | 7        |
| <b>4</b> | <b>RELEVANT IFRS 17 TEXT .....</b>              | <b>9</b> |

# ABBREVIATIONS & DEFINITIONS

| NAME         | DEFINITION                                     |
|--------------|------------------------------------------------|
| <b>ECCIF</b> | Emergency, Chronic, and Critical Illness Fund  |
| <b>IFRS</b>  | International Financial and Reporting Standard |
| <b>PHCF</b>  | Primary Health Care Fund                       |
| <b>POMSF</b> | Public Officers Medical Scheme Fund            |
| <b>SHA</b>   | Social Health Authority                        |
| <b>SHIF</b>  | Social Health Insurance Fund                   |

# 1 INTRODUCTION

## Background

- 1.1 The International Accounting Standards Board ('IASB') introduced IFRS 17 in May 2017 to supersede IFRS 4 which was an interim standard. IFRS 17 insurance contracts establishes the principles for the recognition, measurement, presentation, and disclosure of insurance contracts.
- 1.2 The objective of IFRS 17 is to ensure that an entity provides relevant information that faithfully represent contracts that fall within the scope of the Standard.
- 1.3 The Social Health Authority ('SHA') is a statutory agency established under Section 4 of the Social Health Insurance Act, which was enacted on 22<sup>nd</sup> November 2023 to succeed the National Hospital Insurance Fund (NHIF) as Kenya's primary health care coverage provider. NHIF ceased its operations on **30<sup>th</sup> June 2024**, with its final admissions date being 30<sup>th</sup> September 2024. The transition period, spanning from November 2023 to mid-2024, involved the legal transfer of assets, liabilities and functions. SHA officially commenced its operations on **1<sup>st</sup> October 2024**.
- 1.4 SHA is mandated to pool contributions from individuals, employers, and the government in order to purchase healthcare services from empaneled and contracted healthcare providers and pay for the provision of quality healthcare services to members and their beneficiaries.
- 1.5 SHA operates under the Social Health Insurance Act and manages the four funds:
  - a) Primary Health Care Fund (PHCF);
  - b) Social Health Insurance Fund (SHIF);
  - c) Emergency, Chronic, and Critical Illness Fund (ECCIF)
  - d) Public Officers Medical Scheme Fund (POMSF).

## Purpose

- 1.6 This policy paper defines the principles and procedures for the initial recognition and determination of contract boundaries for contracts managed by the Social Health Authority ("SHA") in compliance with the IFRS 17 standard.

## Objective

- 1.7 This paper aims to ;
  - a) Clarify the criteria and recognition dates for initially recognizing groups of contracts under IFRS 17.
  - b) Elaborate on the concept of a contract boundary within the context of SHA's operations.
  - c) Provide guidance for applying IFRS 17 to the unique characteristics of social health insurance in Kenya, including regulatory pricing, government funding, and periodic contributions.

## Scope

- 1.8 This policy applies to all insurance contracts issued by SHA, that meet the definition of an insurance contract under IFRS 17.
- 1.9 This policy also extends to any other future funds or schemes managed by SHA that fall under the scope of IFRS 17 as insurance contracts.

## 2 INITIAL RECOGNITION

- 2.1 Initial recognition refers to the point at which an insurance contract is first recognized on an entity's statement of financial position.
- 2.2 This occurs when the entity becomes a party to the contract, and the conditions for recognition as per IFRS 17 are met.
- 2.3 As stated in paragraph 25 of the IFRS 17 standard, SHA's insurance contracts shall be recognized at the earliest of the following dates:
  - a) The beginning of the coverage period of the group of contracts
  - b) The date when the first payment from the policyholder is due or received
  - c) The date when a portfolio of insurance contracts becomes onerous.

### Application

- 2.4 For SHA, coverage generally begins when an eligible beneficiary is formally enrolled or registered and becomes legally entitled to access healthcare benefits, provided all other conditions for recognition are met.
- 2.5 This implies that once a member is deemed active and eligible for services based on SHA's operational rules and relevant legislation (e.g., Social Health Insurance Act 2023), the coverage period commences.
- 2.6 In the context of SHA, "contribution due" refers to the legally mandated or agreed-upon contribution (e.g., 2.75% of gross income for formal sector employees, or a fixed amount for informal sector members), irrespective of whether it has been physically received.
- 2.7 For government-funded contributions, recognition occurs when there is a legally binding appropriation, formal budget approval, or a definitive commitment from the government to fund specific beneficiaries for a defined period.
- 2.8 This creates a substantive right for SHA to receive the funds and a substantive obligation to provide services.
- 2.9 A group of insurance contracts is considered "onerous at initial recognition if the sum of the estimated future fulfilment cash flows (i.e., cash outflows for claims and expenses, net of cash inflows from contributions within the contract boundary) exceeds the expected future contributions.

### Recognition Point by Category

| Contributor Type        | Recognition Trigger                                                         |
|-------------------------|-----------------------------------------------------------------------------|
| Formal Sector Employees | Start of coverage month after payroll deduction is due/received             |
| Informal Sector Members | Date of receipt of contribution or start of coverage (whichever is earlier) |
| Government Sponsored    | Date of confirmed allocation or transfer for specific beneficiaries         |

### 3 DETERMINATION OF CONTRACT BOUNDARY

3.1 The contract boundary defines the cashflows that belong to a specific group of insurance contracts. It essentially separates cash flows arising from the legal or contractual rights and obligations of these contracts from other cash flows.

3.2 Paragraph 34 of the Standard defines the boundary of a contract for measurement purposes as follows.

Cashflows are within the boundary of a contract if they arise from substantive rights and obligations that exist during the reporting period in which the entity can compel the policyholder to pay the premiums, or in which the entity has a substantive obligation to provide the policyholder with insurance contract services.

3.3 A substantive obligation for an entity to provide insurance contract services ends, and thus the contract boundary concludes, when:

- a) The entity has the practical ability to reassess the risks of the underlying insurance contract and can set a price or level of benefits that fully reflects these reassessed risks for the period after the reassessment date; or
- b) Both of the following conditions are met:
  - i. The entity has the 'practical ability' to reassess the risk of the portfolio of insurance contracts that contains the contract and can set a price or level of benefits that fully reflects the risk of that portfolio; and
  - ii. The pricing of the contributions up to the reassessment date does not consider the risks that relate to periods after the reassessment date.

3.4 An entity shall not recognize as a liability or as an asset any amounts relating to expected contributions or expected claims outside the boundary of the insurance contract. Such amounts relate to future insurance contracts and are not considered in the measurement of the current contract's liabilities.

3.5 An entity is considered to have the practical ability to set a price at the renewal date, which fully reflects the risks in the contract from that date, when it is not restricted from:

- a) Setting the same price as it would for a new contract issued on that date with the same characteristics as the existing contract;
- b) amending the coverage or terms to be consistent with the price that it will charge; or
- c) setting a price for an individual contract that reflects overall changes in the risks in a portfolio of underlying insurance contracts, even if the price for each individual cedant does not reflect the change in risk for their individual insurance policies.

#### Application to SHA's operating Environment

3.6 In Kenya, SHA's pricing and benefit levels are determined by legislation and regulation.

3.7 This regulatory constraint significantly impacts SHA's "practical ability to reassess risk and set price" as defined in IFRS 17.

3.8 Consequently, for contracts where SHA has limited or no discretion to change contributions or benefits in response to reassessed risks (e.g., standard SHIF contributions and benefits), the contract boundary is typically short-term and tied to the period for which the current legislated terms are firm and beyond which SHA cannot compel payment or has no substantive obligation to provide services. This often implies a monthly or annual rolling contract, renewed with each contribution.

- 3.9 Given that contributions to SHIF are generally monthly or periodic, and the benefits are typically provided on a rolling basis, the default contract boundary for the majority of SHA's insurance contracts (SHIF) is considered to be one month, renewable continuously.
- 3.10 This is because SHA has a substantive obligation to provide services only for the period for which the contribution is due or received, and for subsequent periods, the policyholder has the option not to pay, and SHA has the option to cease coverage (subject to grace periods and regulations).
- 3.11 For beneficiaries whose contributions are funded directly by the government, the contract boundary aligns with the specific fiscal year budget commitments or the coverage period explicitly funded and legally appropriated.
- 3.12 If the government's commitment is for a specific duration (e.g., annual funding for a cohort), the contract boundary extends to that duration. If funding is subject to periodic re-appropriation or review, the boundary is limited to the period of guaranteed funding.
- 3.13 SHA must have clear documentation (e.g., gazette notices, appropriation acts, formal agreements) to support the determination of contract boundaries for these groups.
- 3.14 The table below outlines how contract boundaries shall be determined for each of SHA's main four funds, considering the specific operational and regulatory context.

| Fund Type | Contract Boundary Determination                                                                                                                                    |
|-----------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| SHIF      | One-month rolling contract, renewed with each contribution                                                                                                         |
| PHF       | Same as SHIF unless long-term commitments                                                                                                                          |
| ECCIF     | Up to 12 months of funding is guaranteed; shorter if conditional                                                                                                   |
| POMSF     | Generally aligned to the annual funding/appropriation cycle (up to 12 months), unless funding can be withdrawn, revised or repriced earlier under the scheme terms |

- 3.15 The contract boundary is the period for which the customer has paid, or is obligated to pay, contributions for coverage. This will often be one month, due to the monthly contribution cycle mandated by law.
- 3.16 Future contributions are generally outside the boundary as neither party is strictly compelled to continue the arrangement beyond the current period for all contributors (e.g., informal sector).
- 3.17 While SHA's commitment to Universal Health Coverage (UHC) supports continuity of access to healthcare services, contract boundaries shall nevertheless be assessed in accordance with the underlying legal, contractual, and funding arrangements to ensure accurate financial reporting.
- 3.18 The contract boundary is not static. SHA will periodically reassess the contract boundary, particularly if there are changes in legislation, contribution mechanisms, benefit designs, or funding arrangements that alter SHA's substantive rights or obligations, or its practical ability to reprice or adjust terms.

## 4 RELEVANT IFRS 17 TEXT

Refer to the IFRS 17 Standard available online at the address below:

<https://www.ifrs.org/content/dam/ifrs/publications/pdf-standards/english/2022/issued/part-a/ifrs-17-insurance-contracts.pdf?bypass=on>

This policy has been adopted and signed by the Board of Management of Social Health Authority on this \_\_\_\_\_ day of \_\_\_\_\_ 2026.

Chief Executive Officer:

\_\_\_\_\_

Signature:

\_\_\_\_\_

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## Document Version

May 2026

Version 2.0

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